SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Leuprorelin 22.5 mg: hormone injection every 12 weeks

Advanced disease, or selected high-risk disease with radiotherapy · NCCP 00479 · version 3 · 84-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Schedule
Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Leuprorelin 22.5 mg
GNRH AGONIST

- ☐ Hot flushes, sweating, fatigue and changes in sleep or mood
- ☐ Reduced sex drive, erection difficulties and reduced fertility
- ☐ Loss of bone strength, fracture risk, muscle loss and changes in weight
- ☐ Changes in glucose, cholesterol and cardiovascular risk
- ☐ Heart-rhythm changes; review other medicines that affect the QT interval
- ☐ Initial flare with worsening bone pain or urinary obstruction
- ☐ Injection-site pain or reactions; incorrect preparation can reduce efficacy
- ☐ Depression, allergic reactions and rare raised pressure around the brain

Discuss treatment aim, duration, benefits and uncertainty, alternatives, sexual health and fertility, hot flushes, fatigue, mood, bone and muscle effects, metabolic/cardiovascular risks, interactions, formulation-specific risks, tests and urgent contacts. Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00479 v3.

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